

Station 1: UE and LE COAR

Supine Pectoral Muscle Stretch

Possible Clinical Indications: Upper Crossed Syndrome, Protracted Shoulders, Shoulder pain

Possible TART Findings/Diagnosis: Rt/Lt/Bilateral Hypertonic Pectoral Muscles

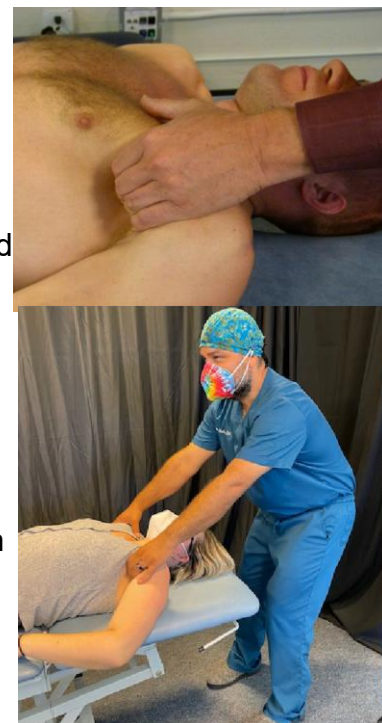
Example Diagnosis: Bilateral Hypertonic Pectoralis Major Muscles

Patient Position: Supine

Physician Position: Standing at head of table

Treatment:

1. The physician identifies the anterior axillary fold bilaterally and gently slides 2-4 fingers, depending on the physician and patient body habitus, under each pectoral muscle
2. The physician gently lifts the pectoral muscles anteriorly to tension and asks the patient to breathe in (this will lift their chest anteriorly and superiorly)
3. The physician then follows this motion of the breath superiorly with their contact under the pectoral muscles
4. The physician maintains this contact and position as the patient exhales, causing a stretch of the pectoral muscles
 - a. This can be repeated multiple times to further engage and stretch the muscles
 - b. The physician can lean posteriorly to further enhance the stretch
5. The physician continues to hold anterior traction until a release is felt
6. The physician gently releases contact with the pectoral muscles
7. Reassess



Supine Latissimus Dorsi and Teres Major Stretch

Possible Clinical Indications: Upper Crossed Syndrome, Protracted Shoulders, Shoulder pain

Possible TART Findings/Diagnosis: Rt/Lt/Bilateral Hypertonic Latissimus Dorsi and Teres Major Muscles

Example Diagnosis: Bilateral Hypertonic Latissimus Dorsi and Teres Major Muscles

Patient Position: Supine

Physician Position: Standing or sitting at head of table

1. The physician identifies the posterior axillary fold bilaterally and gently slides 2-4 fingers, depending on the physician and patient body habitus, into the posterior axilla
 - a. Use of curled fingers when placing hands can avoid pinching and/or sliding on the tissues during insertion
2. The physician gently lifts superiorly the posterior axillary fold engaging the latissimus dorsi and teres major muscles to tension
3. The physician maintains this contact and position causing a stretch of the muscles
 - a. Respiratory assistance can be used to further engage and stretch the muscles
 - b. The physician can lean posteriorly to further enhance the stretch
4. The physician continues to hold superior traction until a release is felt



5. The physician gently releases contact with the posterior axillary fold, latissimus dorsi and teres major muscles
6. Reassess

BONUS: You can alternatively perform muscle inhibition to this area

Patient Position: Supine

Physician Position: Seated at the side of the bed

1. The physician contacts the latissimus dorsi and teres major muscles between their thumb and finger pads
2. The physician applies pressure between their thumb and finger pads, matching the tissue tension
3. The physician holds until a release is felt
4. Reassess



Unilateral Scapulothoracic Balanced Tension

Possible Clinical Indications: Upper Crossed Syndrome, Protracted Shoulders, Shoulder Pain

Possible TART Findings/Diagnosis: Rt/Lt/Bilateral decreased shoulder ROM, Rt/Lt/Bilateral paraspinal hypertonicity, Rt/Lt/Bilateral Glenohumeral decreased motion anterior/posterior/lateral

Example Diagnosis: Lt Shoulder decreased ROM

Patient Position: Seated

Physician Position: Standing or seated at side of patient

Treatment:

1. The physician's more anterior hand contacts the Lt anterior chest wall with fingers draping over the clavicle and thumb hooking underneath the pectoralis minor muscle
2. The physician's more posterior hand contacts the posterior aspect of the Lt scapula with the fingers contacting the spine of the scapula and the thumb into the subscapular space
 - a) The pad of the thumb will contact subscapularis and the tip of the thumb towards the ribs
 - b) The patient's Lt arm is then relaxed at their side
3. The physician uses both hands to move the Lt scapula into protraction/retraction, elevation/depression, CW/CCW planes until a balance in the shoulder girdle tissues is appreciated
4. The physician holds this position until a release is felt
5. Reassess



Bilateral Scapular Balanced Tension

Possible Clinical Indications: Upper Crossed Syndrome, Protracted Shoulders, Shoulder Pain

Possible TART Findings/Diagnosis: Rt/Lt/Bilateral decreased shoulder ROM, Rt/Lt/Bilateral paraspinal hypertonicity

Example Diagnosis: Bilateral scapulothoracic decreased ROM

Patient Position: Supine

Physician Position: Standing or seated at head of the table

Treatment:

1. The physician places a hand under each scapula, proceeding to bring each scapula into a point of balanced tension one at a time. Start with the most dysfunctional side and move the scapula into protraction/retraction, elevation/depression, CW/CCW planes until a balance is appreciated
2. The physician holds this position until a release is felt
 - a) Release can occur in stages, one scapula releasing first then the other
3. Reassess



Supinated Forearm MET

Possible Clinical Indications: Forearm pain, decreased ROM, muscular hypertonicity

Possible TART Findings/Diagnosis: R/L Supinated forearm (pronation restriction)

Example Diagnosis: Right supinated forearm

Patient Position: Seated with elbow flexed at 90 degrees and adducted

Physician Position: Standing or seated in front or at the side of the patient

Treatment:

1. The physician holds the patient's elbow with one hand and with the other hand, holds the patient's wrist/hand as in a handshake
 - a) The physician can use their index finger to cross the wrist joint to eliminate carpal bone motion at the wrist while testing and/or performing the technique
2. The physician slowly pronates the forearm until they begin to feel the feather's edge of the restrictive barrier
3. The physician then instructs the patient to turn their hand in the opposite direction (supination), against the physician's unyielding counterforce
4. The patient maintains that effort for 3-5 seconds, until the physician instructs them to relax
5. During relaxation the physician maintains the counterforce, so the position of the hand does not change
6. The physician waits for 2-3 seconds, being sure the patient has stopped the effort completely and then repositions into the feather's edge of the new restrictive barrier
7. The physician repeats these steps until no further change in the restrictive barrier is noted; 3-5 times
8. The physician performs a final stretch into the restrictive barrier following the final repetition
9. Reassess



Pronated Forearm MET

Possible Clinical Indications: Forearm pain, decreased ROM, muscular hypertonicity

Possible TART Findings/Diagnosis: R/L Pronated forearm (supination restriction)

Example Diagnosis: Right pronated forearm

Patient Position: Seated with elbow flexed at 90 degrees and adducted

Physician Position: Standing or seated in front or at the side of the patient

Treatment:

1. The physician holds the patient's elbow with one hand and with the other hand, holds the patient's wrist/hand as in a handshake
 - a) The physician can use their index finger to cross the wrist joint to eliminate carpal bone motion at the wrist while testing and/or performing the technique
2. The physician slowly supinates the forearm until they begin to feel the feather's edge of the restrictive barrier
3. The physician then instructs the patient to turn their hand in the opposite direction (pronation), against the physician's unyielding counterforce
4. The patient maintains that effort for 3-5 seconds, until the physician instructs them to relax
5. During relaxation the physician maintains the counterforce, so the position of the hand does not change
6. The physician waits for 2-3 seconds, being sure the patient has stopped the effort completely and then repositions into the feather's edge of the new restrictive barrier
7. The physician repeats these steps until no further change in the restrictive barrier is noted; 3-5 times
8. The physician performs a final stretch into the restrictive barrier following the final repetition
9. Reassess

**Radiocarpal MET**

Possible Clinical Indications: Wrist pain, decreased ROM, muscular hypertonicity of the forearm

Possible TART Findings/Diagnosis: R/L Flexed/Extended/ABducted/ADducted

Example Diagnosis: Right Flexed Wrist (Radiocarpal joint)

Patient Position: Seated with elbow flexed at 90 degrees and adducted

Physician Position: Standing or seated in front or at the side of the patient

Treatment:

1. The physician holds the patient's elbow with one hand and with the other hand, holds the patient's wrist/hand
2. The physician slowly extends the wrist until they begin to feel the feather's edge of the restrictive barrier
3. The physician then instructs the patient to push their wrist/hand in the opposite direction (flexion), against the physician's unyielding counterforce
4. The patient maintains that effort for 3-5 seconds, until the physician instructs them to relax
5. During relaxation the physician maintains the counterforce, so the position of the hand does not change
6. The physician waits for 2-3 seconds, being sure the patient has stopped the effort completely and then repositions into the feather's edge of the new restrictive barrier
7. The physician repeats these steps until no further change in the restrictive barrier is noted; 3-5 times
8. The physician performs a final stretch into the restrictive barrier following the final repetition
9. Reassess



In all types of SD in this region (R/L Flexed/Extended/ABducted/ADducted) ask the patient to move their hand in the opposite direction and perform the MET sequence, 3-5 times, until no further change is noted, perform a final stretch and reassess

Carpal Tunnel Release (Flexor Retinaculum/Transverse Carpal Ligament Release)

Possible Clinical Indications: Wrist pain, decreased ROM, swelling

Possible TART Findings/Diagnosis: R/L myofascial restriction

Example Diagnosis: Left myofascial restriction

Patient Position: Supine or seated

Physician Position: Standing or seated in front or at the side of the patient

Treatment:

1. The physician holds the patient's wrist and hand with the finger pads wrapped around the dorsum of the wrist/hand and the thumb pads on the volar (anterior) aspect of the wrist at the level of the flexor retinaculum
2. The physician maintains the natural arch of the carpal bones, while applying a lateral spread of the flexor retinaculum in a rhythmic motion
3. The physician continues until a release or softening is felt
4. Reassess

**Popliteal Fossa Release (Soft Tissue/Lymphatics)**

Possible Clinical Indications: LE swelling, knee pain, recent illness (lymphatic progressive treatment)

Possible TART Findings/Diagnosis: Distal hamstring hypertonicity, fluid congestion and/or bogginess

Example Diagnosis: L popliteal fossa bogginess

Patient Position: Supine

Physician Position: Standing, seated at side of bed

Treatment:

1. The physician contacts the left popliteal fossa, with finger pads wrapping around the popliteal fossa
2. The physician's finger pads should be cupping the hamstring attachments, but also attending to any congestion and/or bogginess/fluid in the popliteal region
3. The physician applies lateral traction to the left distal hamstring's, spreading the popliteal fossa, kneading the distal hamstrings
4. The physician rhythmically kneads until a release is felt
 - a) Optionally, the physician can hold the hamstrings apart, holding tension, until a release is felt
5. Reassess

Plantar Fascia Release- Soft Tissue

Possible Clinical Indications: Lower extremity pain, gait disturbances, Plantar tension, dropped arch, dropped tarsal bone, plantar pain

Possible TART Findings/Diagnosis: R/L/Bilateral plantar tension, R/L/Bilateral dropped arch

Example Diagnosis: L plantar fascia restriction

Patient Position: Supine

Physician Position: Standing or seated at the end of the table

Treatment:

1. The physician places one hand on the dorsum of the left foot, the other hand makes a fist and contacts the plantar surface with the lateral edge of their 5th digit
2. The physician provides a rhythmic force into the plantar fascia in a superior to inferior motion with the lateral edge of the 5th digit, while providing a counter force with their hand on the dorsum of the foot
3. The physician provides a continuous rhythmic force until a release is felt
4. Reassess



Arch Springing- Soft Tissue

Possible Clinical Indications: Plantar tension, dropped arch, dropped tarsal bone, plantar pain

Possible TART Findings/Diagnosis: R/L/Bilateral plantar tension, R/L/Bilateral dropped arch, R/L/Bilateral dropped cuboid and/or navicular

Example Diagnosis: R plantar tension

Patient Position: Supine

Physician Position: Standing or seated at the end of the bed

Treatment:

1. The physician contacts the foot to be treated and places their cephalad hand medially and their caudad hand more lateral
2. The physician then makes a wringing motion from medial to lateral (external rotation) with their cephalad hand, while making a wringing motion lateral to medial (internal rotation) with their caudad hand
3. The physician additionally uses their thumb pads to rhythmically lift and/or spring into the plantar surface while performing the wringing motion as above
4. The physician continues these motions until a release is felt
5. Reassess



Internal and External Rotation of the Tibia- MET

Possible Clinical Indications: Lower extremity pain, gait disturbances

Possible TART Findings/Diagnosis: Rt/Lt/Bilateral internally rotated Tibia, Rt/Lt/Bilateral externally rotated Tibia

Example Diagnosis: Rt internally rotated Tibia

Patient Position: Seated

Physician Position: Standing or seated at the side of the patient

Treatment:

1. The physician positions the patient's Rt tibia into external rotation while holding at the tibial plateau and at the medial/lateral malleoli
2. The physician asks the patient to contract their muscles to back toward neutral, while maintaining an unyielding counterforce (physician force=patient force) and asks the to hold for 3-5 seconds
3. The physician instructs the patient relax completely waiting for seconds
4. The physician then repositions the right tibia into the feather's edge of the barrier repeating steps 2 and 3, until a release is felt (typically 2-5 times)
5. Reassess



move
patient
1-2

Ankle Plantarflexion and Dorsiflexion- MET

Possible Clinical Indications: Lower extremity pain, gait disturbances

Possible TART Findings/Diagnosis: Rt/Lt/Bilateral plantarflexed, Rt/Lt/Bilateral dorsiflexed

Example Diagnosis: Lt plantarflexed ankle

Patient Position: Supine

Physician Position: Standing or seated at the end of the bed

Treatment:

1. The physician places the Lt ankle into dorsiflexion, placing one hand behind the ankle for support and the other on the dorsal surface
2. The physician asks the patient to contract their muscles to move back toward neutral (toward plantarflexion), while maintaining an unyielding counterforce (physician force=patient force) and ask the patient to hold for 3-5 seconds
3. The physician instructs the patient to relax completely waiting for 1-2 seconds
4. The physician then repositions the right tibia into the feather's edge of the barrier repeating steps 2 and 3, until a release is felt (typically 2-5 times)
5. Reassess



Station 2: FPR and LBP COAR

Cervical FPR -Superficial Muscles

Clinical Indications: Neck Pain, Headaches, Whiplash

Possible TART Findings: Cervical TART changes

Diagnosis: Hypertonicity- SCM, Scalenes, Suboccipitals, Rectus Capitus etc

Patient position: Supine

Physician position: Seated

1. Screen/scan the cervical musculature for areas of tissue texture abnormalities, select an area of muscle hypertonicity
2. Patient supine with head and neck on the table **or** off the end of the table, resting on a pillow in the doctor's lap.
3. Monitor the area of dysfunction while cradling the neck in the hand opposite the dysfunction (cradle with right hand for left-sided dysfunction). Use the finger pad as a monitor and fulcrum for treatment.
4. Use the opposite hand on the patient's head to **straighten the cervical lordosis into a neutral spine position**
5. Direct a compressive force through the hand on the head to the dysfunctional region sufficient to produce tissue softening in the hypertonic muscle
6. Further shorten the muscle(s) with positioning of flexion/extension, rotation, and/or sidebending. **The specific movement will be dependent upon which muscle is being treated.**
7. Maintain the treatment position for about 3-5 seconds and monitor for further change in the tissues
8. Return to a neutral position and re-assess. A gentle stretch in the direction of the previous restriction can be added.
9. Reassess



Cervical FPR

Clinical Indications: Neck Pain, Headaches, Whiplash

Possible TART Findings: Cervical TART changes

Diagnosis: C3 ERRSR

Patient position: Supine

Physician position: Seated

1. The Physician is seated at the head of the table with the patient supine.
2. Cradle the patient's neck with the middle finger, monitoring the articular pillar of the dysfunctional segment. The thumb can be on the contralateral articular pillar. Support the neck with the rest of the hand.
3. Passively flex the neck to flatten the cervical lordosis. To obtain a neutral spine position, it may be necessary to place the patient far enough up the table to have the head and neck off the table and supported in the physician's lap, possibly with a pillow
4. Induce a compressive force through the head into the neck to the level of C3
5. Maintain compressive force and move the segment into the positions of ease in all planes, C3 ERRSR
6. Expect nearly immediate release of the articular restriction
7. Hold for about 3-5 seconds, return to the original position
8. Expect nearly immediate release of the articular restriction, and monitor for tissue change
9. Reassess



Thoracic FPR: Upper Thoracic Spine (T1-T4)

Clinical Indications: Back pain, neck pain, shoulder pain, viscerosomatic dysfunction

Possible TART Findings: Thoracic TART changes

Diagnosis: T3 ERLSL

Patient position: Seated

Physician position: Standing on the Ipsilateral side

1. The physician stands behind the seated patient on the side of the dysfunction
2. Monitor the affected segment with the hand closest to the patient at the T3 transverse processes
3. Gently contact the patient's head with either an open hand or utilizing a "hat hold" contact through the bicep and forearm. It may be helpful to stabilize the position by allowing the doctor's elbow to rest on the patient's ipsilateral shoulder if the neck can be kept in midline.
4. Obtain a neutral spine position. The patient may assist with positioning by sitting up straight, lifting the sternum, or moving the shoulders slightly posteriorly. The patient must then relax.
5. Add a compressive force directed to the segment
6. Maintain compressive force and move the segment into the position of ease in all planes - T3 ERLSL.
7. Expect nearly immediate release of the articular restriction, and monitor for tissue change
8. Hold for about 3-5 seconds, return to the original position
9. Reassess



Alternate Position

Thoracic FPR: Lower Thoracic Spine (T5-T12)

Clinical Indications: Back pain, viscerosomatic dysfunction, etc.

Possible TART Findings: Thoracic TART changes

Diagnosis: T8 ERLSL

Patient position: Seated

Physician position: Standing on the Ipsilateral side

1. The physician stands behind the seated patient on the side of the dysfunction
2. Monitor the affected segment, T8, with the hand closest the patient at the transverse processes.
3. Place the physician's axilla or bicep over patient's shoulder and hand on opposite shoulder, as in Muscle Energy (Alternate:). Place the physician's forearm behind the patient's neck with the elbow on the shoulder side of the dysfunction and the hand on the opposite shoulder.
4. Instruct the patient to sit up straight to flatten the thoracic kyphosis. You may need to have the patient push out their chest or slump to further flatten the thoracic spine
5. Induce an even compressive force through the physician's axilla and hand (or forearm) to T8 of the dysfunction with the vector straight down parallel to the spine to the dysfunctional segment.
6. Maintain compressive force and move the segment into the position of ease in all planes - T8 ERLSL.
7. Expect nearly immediate release of the articular restriction, and monitor for tissue change
8. Hold for about 3-5 seconds, return to the original position
9. Reassess



Thoracic FPR: Extended Somatic Dysfunction Prone Position**Clinical Indications:** Back pain, viscerosomatic dysfunction, etc.**Possible TART Findings:** Thoracic TART changes**Diagnosis:** T4 ERRSR**Patient position:** Prone**Physician position:** Standing on the contralateral side

1. The physician stands beside the prone patient, contralateral to the side of the dysfunction
2. Monitor the T4 posterior transverse process on the right with the hand closest to the patient
3. Contact the patient's opposite shoulder with the other hand;
4. Check the thoracic kyphosis for a neutral spine position. **If needed, add a pillow under the patient's head and neck or upper chest and/or abdomen**
5. Induce a compressive force first medial toward the spine, then inferiorly towards the patient's feet to provide the activating force **and side bending**
6. Maintain compressive force with traction on the shoulder. Add extension and rotation by lifting the patient's shoulder off the table by straightening the physician's body until motion is felt at the monitoring finger
7. Expect nearly immediate release of the articular restriction, and monitor for tissue change
8. Hold for about 3-5 seconds, return to the original position
9. Reassess

**NOTE:** The prone position is not suitable for flexion thoracic dysfunctions**Lumbar FPR: Lateral Recumbent Position****Clinical Indications:** Back pain**Possible TART Findings:** Lumbar TART changes**Diagnosis:** L5 ERLSL**Patient position:** Lateral recumbent**Physician position:** Standing facing the patient

1. The physician stands beside the table facing the patient
2. The patient is **side lying with side of posterior transverse process up** (lying on right side, in this case) and the right arm under the patient's head to induce left sidebending.
3. Flex the hips to bring the lumbar spine into a neutral spine position, facets neither open nor closed
4. Monitor L5 with the cephalad hand at the spinous process.
5. Flex the hips more to introduce flexion felt at the monitoring finger and then "unflex" to extend L5
6. Introduce gentle left rotation from above to L5 using the cephalad hand at the patients shoulder.
7. Accentuate the L5 rotation left by gently rotating the pelvis and sacrum to the right from below



- Accentuate sidebending **with compression** from the caudad forearm at the gluteal region (side bending from below). This is a case where we **add the compression after** the positioning of ease.

Note: These motions are less than used for MET feathers edge barrier

- Expect nearly immediate release of the articular restriction, and monitor for tissue change
- Hold for about 3-5 seconds, return to the original position
- Reassess

Note: that the initial lateral recumbent position is opposite to the way you have treated lumbar MET and HVLA!

We position to the ease and so the posterior transverse process is up, NOT down, allowing you to position from above and below the SD segment as you have previously done for MET and HVLA!

For a type 1 Lumbar diagnosis, L5 N RISR, Position the patient in the same way, except alter the side bending by adding a pillow above the patient's right hip to induce right sidebending, and have the patient support their head with their right **arm bent**, avoiding inducing left sidebending.

Still Technique: 1st rib

Clinical Indications: Neck pain, back pain, shoulder pain, thoracic outlet syndrome

Possible TART Findings: Elevated 1st rib

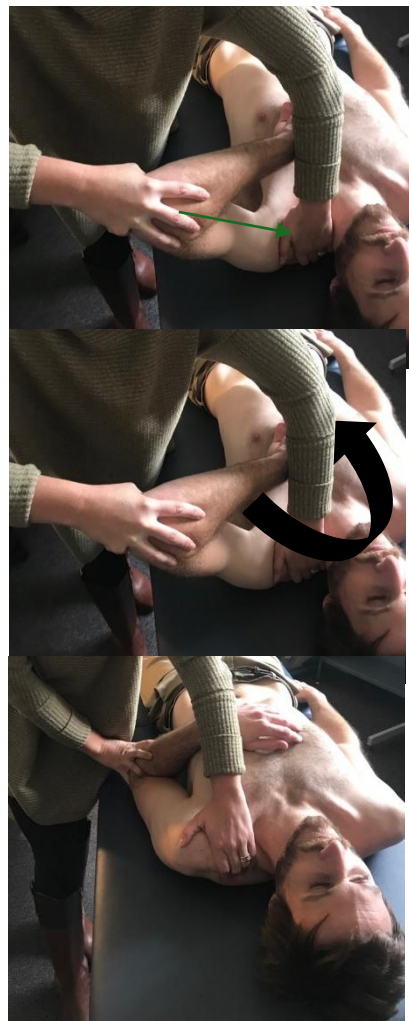
Diagnosis: L5 ERLSL

Patient position: Supine

Physician position: Standing facing the patient on the ipsilateral side.

- Diagnose the first rib for either non-physiologic rib elevation or inhalation somatic dysfunction
- The physician stands on the side of the elevated or inhaled first rib
- Place a finger of the hand closer to the patient on the posterior aspect of the first rib, at the area of greatest tension. This contact is primarily for monitoring. The patient's forearm and **supine** wrist are placed on their chest. The physician's forearm of the monitoring hand is placed above the patient's wrist. This will provide a mechanical fulcrum for some subsequent maneuvers.
- Contact the patient's elbow with the other hand. Flex the shoulder to about 90 degrees and abduct slightly from a vertical position to a place of maximal softening under the monitoring fingers.
- Introduce a compressive force through the patient's shoulder towards the affected first rib to further soften the tissues.
- Additional shoulder flexion will bring the patient's forearm against the student doctor's opposite forearm and produce some internal rotation of the patient's shoulder. Monitor for softening and relaxation at the first rib.
- Maintain this position of ease for about 3-5 seconds
- While maintaining the compressive force and internal rotation, move the patient's shoulder into adduction with further internal rotation, completing the motion with medial circumduction, finishing in a neutral position alongside the torso.
- Re-assess the first rib

Note: The Physician's Forearm stabilizes the wrist and assists with guiding the patient's arm through the circumduction and back to neutral.



Note: The Still Technique is a combination move of an initial FPR neutral/activating force and ease treatment, but then continues in a direct manner to and through the articulation barrier.

Iliolumbar Ligament Balanced Ligamentous Tension (BLT)

Possible Clinical Indications: Low back pain, sacroiliac joint dysfunction, pelvic asymmetry, hypertonic quadratus lumborum

Possible TART Findings/Diagnosis: Point tenderness or tension between L4 and the iliac crest; restricted pelvic or lumbar motion (patient may be prone for assessment)

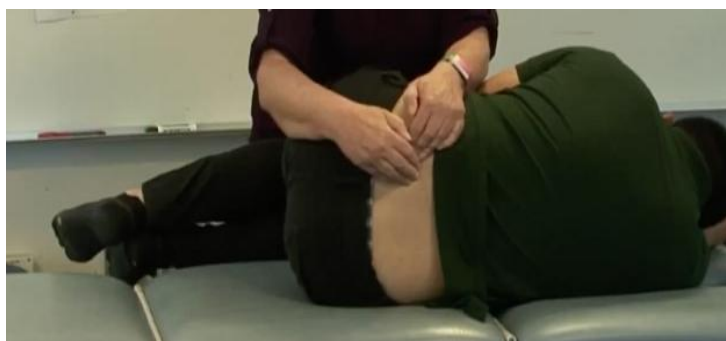
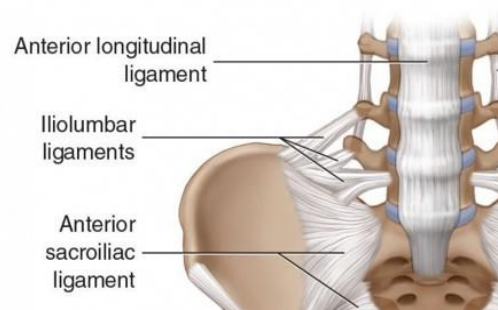
Example Diagnosis: Left iliolumbar ligament tension

Patient Position: Lateral recumbent with the dysfunctional side up

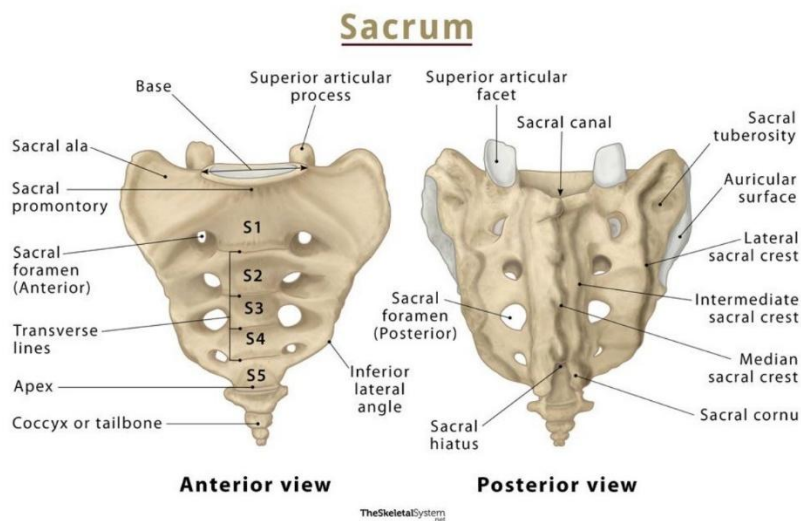
Physician Position: Standing facing the patient's anterior body

Treatment:

1. The physician assesses the iliolumbar ligament in the **prone** position, palpating between L4 and the iliac crest bilaterally for tenderness or tension.
2. The patient is positioned in **lateral recumbency**, with the dysfunctional side up.
3. The **cephalad hand** monitors the iliolumbar ligament at the point of greatest tension.
4. The patient bends their knees, and the **caudad hand/arm** is used to guide the lower extremities into a position of **ease** (position of least tension) as sensed by the monitoring hand.
5. The physician uses their body to support and maintain the legs in position.
6. The **caudad forearm** contacts the greater trochanter and gently applies **compressive force** toward the monitoring hand over the ligament.
7. Both hands are **fine-tuned** to achieve a balanced tension point throughout the iliolumbar ligaments.
8. The patient is instructed to take a few **deep breaths**, serving as the activating force.
9. The patient is returned to neutral, and the physician **reassesses** the iliolumbar ligament for reduced tenderness and tension in the **prone** position.



Sacral Segmental Balanced Ligamentous Tension (BLT)



Sacrum

Possible Clinical Indications: Sacral dysfunction, low back pain, pelvic asymmetry, SI joint dysfunction, postural imbalance

Possible TART Findings/Diagnosis: Localized tenderness or restricted motion at sacral segments or poor sacral springing

Example Diagnosis: S1 FRLSL (S1 left posterior at that segment)

Patient Position: Prone

Physician Position: Standing or seated beside the patient



Treatment:

1. The physician contacts the transverse processes of the sacral base (S1) using their thumbs and assesses for tenderness, asymmetry, or segmental restriction at each sacral level.
2. The physician places their ipsilateral thumb and contralateral 2nd digit over the cephalad segment while stabilizing the sacrum with the ipsilateral thumb and contralateral 2nd digit of the caudad hand.
3. The position is fine-tuned to achieve balanced tension, and the patient is held in this position

until a release is felt.

4. The sacral segments are reassessed for improved mobility and decreased tenderness.

Seated Still Technique for Lumbar Region

Possible Clinical Indications: Low back pain, sacroiliac joint dysfunction, segmental lumbar dysfunction, somatic dysfunction contributing to postural imbalance or gait disturbance

Possible TART Findings/Diagnosis: Localized tissue texture changes, asymmetry, segmental restriction, tenderness in lumbar vertebrae

Example Diagnosis: L3 F RLSL or Left SI joint restriction

Patient Position: Seated, arms crossed with elbows together in front of chest

Physician Position: Standing behind the patient



Treatment:

1. The physician diagnoses a lumbar somatic dysfunction (e.g., L3 N RLSL) with the patient seated or prone.
2. The physician places one hand to monitor the dysfunctional segment, and instructs the patient to cross their arms in front of their chest, bringing their elbows together.
3. The physician induces ease by passively adjusting flexion/extension, sidebending, and rotation (above and below the segment) to find the position of greatest ease at the monitored segment.
4. Once ease is obtained, the physician adds either:
 - Compression: Arm across patient's shoulders, pressing downward in a vector toward the dysfunctional segment.
 - Traction: Hand under elbows, gently lifting upward to provide axial traction without inducing extension.
5. While maintaining the compression or traction, the physician guides the patient through a gentle arc of motion from ease, through the restrictive barrier, and back to neutral.

Iliopsoas Facilitated Positional Release (FPR)

Possible Clinical Indications: Anterior pelvic tilt, hypertonic iliopsoas, low back pain, lumbar somatic dysfunction

Possible TART Findings/Diagnosis: Hypertonic lumbar paraspinals or psoas; restricted lumbar extension

Example Diagnosis: Left iliopsoas hypertonicity

Patient Position: Supine

Physician Position: Standing on the ipsilateral side

Treatment:

1. The physician places the monitoring hand under the lumbar paraspinals at the junctional level of dysfunction.
2. With the operating arm or torso, the physician grasps the patient's ipsilateral leg and applies gentle axial compression through the femur toward the monitoring hand.
3. Once a softening is felt in the monitored tissues, the physician instructs the patient to actively straighten the leg against resistance all the way down to the table.
4. Once the leg is on the table, the physician maintains contact and applies a gentle final tractional tug.
5. Reevaluate the original structural diagnosis and response to treatment.
6. Recheck the dysfunctional segment for tissue and motion changes.

